

[REDACTED]

Histopathology Report		Biopsy No:	Corrected
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Received Date/Time	[REDACTED]	Final	
Reporting Information	Date/Time : [REDACTED] Histopathologist : [REDACTED] Laboratory : [REDACTED]	Final	Updated
Consultant-In-Charge	[REDACTED]	Final	
Submitting Physician	[REDACTED]	Final	
Histopathology Report		Final	Updated

ADDENDUM

ADDENDUM TO HISTOPATHOLOGY REPORT [REDACTED]

Histology shows a confluent, nodular-to-diffuse, patternless infiltrate of lymphomatous medium-to-large cells "open" nuclei, granular chromatin, spotty karyorrhexis, relatively inconspicuous nucleoli and ample cytoplasm, in one focus compartmentalised by the sclerotic strands (Block A1), entrapping skeletal muscle with focal secondary regenerative multinucleation, walled partially by fibrosis (Block A2). Atrophic thyroid follicles amidst a background of lymphocytic thyroiditis are noted focally (Block A1).

The lymphomatous infiltrate is based almost entirely on CD21-immunoreactive follicular dendritic meshworks that maintain circumscription in nodular areas but are extensively expanded and disrupted in diffuse areas. The lymphoma is diffusely positive for pan-B cell marker CD20, with coexpression of germinal centre-associated CD10, while the cell proliferation fraction is fairly uniform around approximately 70-80% by Ki-67 immunolabelling, highlighting moderate anisokaryosis amongst medium-to-large cells, as well as an intermingled, diminutive centrocytic component discernible focally, in keeping with a component of WHO Grade 3A follicular lymphoma, hence contextually and biologically excluding Burkitt's lymphoma despite neither overexpression of antiapoptosis protein bcl-2, nor upregulation of late-to-postgerminal centre-associated MUM.1, both of which appear largely accounted for by physiological expression in CD3-positive, reactive small T-lymphocytes that disclose perifollicular accentuation. Immunostaining for terminal deoxynucleotidyl transferase is negative within the lymphomatous population, highlighting only physiological expression in a few, dispersed haematogones, ruling out a lymphoblastic neoplasm. Negativity for cyclin D1, except for physiological expression in scattered thyrofollicular epithelial, histiocytic and activated endothelial nuclei, excludes a transformed mantle cell lymphoma.

UPDATED DIAGNOSIS:

RIGHT THYROID MASS; INCISIONAL BIOPSIES:

LARGE B-CELL LYMPHOMA, DIFFUSE AND FOLLICULAR (WHO GRADE 3A).

Pathologist : [REDACTED]